Project Dunecrest

Transaction Teaser – Dunecrest Continuation Fund

September 2026

Kessport Frontier Managers Limited – General Partner of Kessport Frontier Fund II

Multi-asset GP-led continuation of Arvenna Health Group and Tallowick Consumer Products

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Section 1

Transaction Overview

Executive Summary

Kessport Frontier proposes a continuation vehicle to acquire Fund II's last two portfolio companies and offer Fund II LPs a liquidity or roll election

Transaction Summary

- Kessport Frontier Managers Limited (the "Sponsor") is pursuing a multi-asset transaction (the "Transaction" or "Project Dunecrest") for the last two remaining investments of Kessport Frontier Fund II ("Fund II"): Arvenna Health Group ("AHG") and Tallowick Consumer Products ("Tallowick", together the "Companies" or the "Portfolio").
- A newly formed vehicle, Dunecrest Continuation Fund (the "Continuation Vehicle" or "CV"), will acquire Fund II's 58% interest in AHG and 74% interest in Tallowick at a purchase price of $143.6M (93.3% of the $153.9M Reference Date NAV), alongside $10.2M of committed follow-on capital for the add-on programmes of both Companies.
- Total CV commitments of $167.7M are assumed, comprising $145.1M of funded capital at closing (purchase price plus $1.4M of buy-side transaction fees), $10.2M of follow-on equity and $12.4M of management fees and expenses drawn over the life of the CV.
- Fund II limited partners may elect to sell for cash, roll their interest into the CV on the same terms as new investors, or a combination; the base case assumes 41% of the funded capital is rolled ($59.5M), $5.2M of GP economics is reinvested, the GP commits $2.9M out of pocket and $77.5M is provided by new investors.

Portfolio Snapshot

Company	Description	Holding TEV	EV / LTM EBITDA	Fund II stake	NAV	% of NAV
Arvenna Health Group	Largest integrated private hospital group in Country A; 4 hospitals, diagnostics arm, ~620 beds	$110.4M	8.0x	58%	$49.0M	31.8%
Tallowick Consumer Products	Branded packaged-food and household-products manufacturer in Country B; 3 plants, ~35% share of core category	$141.5M	7.0x	74%	$105.0 M	68.2%
Total					**$153.9 M**	**100.0%**

Transaction Objectives

- 1. Provide optional liquidity to Fund II investors and wind up the fund
  - Fund II (2017 vintage) is in its tenth year; the two remaining assets represent the bulk of remaining NAV and cannot be realised at full value within the fund term.
- 2. Extend the holding period to unlock the upside in both Companies
  - AHG's radiotherapy build-out, bed expansion and add-on pipeline and Tallowick's premium-mix migration are expected to lift combined EBITDA from $36.9M in FY2026E to $55.2M in FY2030E.
- 3. Acquire the interests at a modest discount to NAV
  - Purchase prices at 94% (AHG) and 93% (Tallowick) of Reference Date NAV, with NAV set at 8.0x and 7.0x LTM Q2'26 EBITDA respectively; co-investors (Fund III, DFI Partner 1, DFI Partner 2, Tallowick management) retain their stakes.
- 4. Provide unfunded capital for growth
  - $10.2M of follow-on equity is committed pro rata to the CV ownership to fund the identified acquisitions; other shareholders fund their share.

$167.7M Total CV commitments	**2.44x** Base-case gross MoIC
20.2% Base-case gross IRR	**2.00x / 16.7%** Net MoIC / net IRR to new LPs

Notes: Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026; USD M unless indicated; LCA/LCB at 0.235/0.0125 USD. No assurance projections will be realised. (1) Assumes no incremental exposure to the co-investor stakes.

Transaction Structure

A single closing transfers Fund II's two interests into the Continuation Vehicle; rolling LPs, new LPs and the GP capitalise the vehicle

Transaction Description

- A. Continuation Vehicle
  - Dunecrest Continuation Fund, a newly formed limited partnership managed by the Sponsor, acquires Fund II's interests in AHG and Tallowick at closing (31 December 2026) and commits follow-on capital.
  - Capitalised by the GP's out-of-pocket commitment ($2.9M, 2% of funded capital), reinvested GP economics ($5.2M), rolling Fund II LPs and new investors.
- B. New Limited Partners
  - Acquire the interests of selling Fund II LPs net of fees and expenses; base case $77.5M of funded capital (53.4%).
  - Same terms, same fee and carry as rolling LPs; the CV constitutes a single class of interests.
- C. Existing Fund II LPs
  - Election period of 20 business days after the Fund II LPAC status-quo option is confirmed: (i) sell for cash at the purchase price, (ii) roll into the CV, or (iii) a combination.
  - Fund II is wound up after closing; escrows from prior exits are distributed under the existing LPA.

Funded capital at closing by source



Structure Overview

Dunecrest Continuation Fund (A)

Rolling Fund II LPs (C)	New LPs (B)	GP commitment & reinvested economics	Fund II (seller)

Flow	From	To	Amount
Purchase price – AHG (58% stake)	Continuation Vehicle	Fund II	$46.0M
Purchase price – Tallowick (74% stake)	Continuation Vehicle	Fund II	$97.6M
Buy-side transaction fees (1.0%)	Continuation Vehicle	Advisers	$1.4M
Transfer of ownership	Fund II	Continuation Vehicle	AHG 58% / Tallowick 74%
Cash proceeds to selling LPs	Fund II	Selling LPs	$78.9M
Rolled interests (in kind)	Fund II LPs	Continuation Vehicle	$59.5M
Follow-on commitments	CV investors	Companies (2027–2030)	$10.2M

Ownership after closing

Shareholder	AHG	Tallowick
Dunecrest Continuation Fund	58%	74%
Kessport Frontier Fund III	12%	–
DFI Partner 1 / DFI Partner 2	15% / 15%	–
Management	–	26%
Total	**100%**	**100%**

Notes: Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026; USD M unless indicated; LCA/LCB at 0.235/0.0125 USD. No assurance projections will be realised.

Portfolio Overview & Continuation Fund Returns

The Portfolio is held at 2.60x cost; the CV base case returns 2.44x / 20.2% gross on total investment

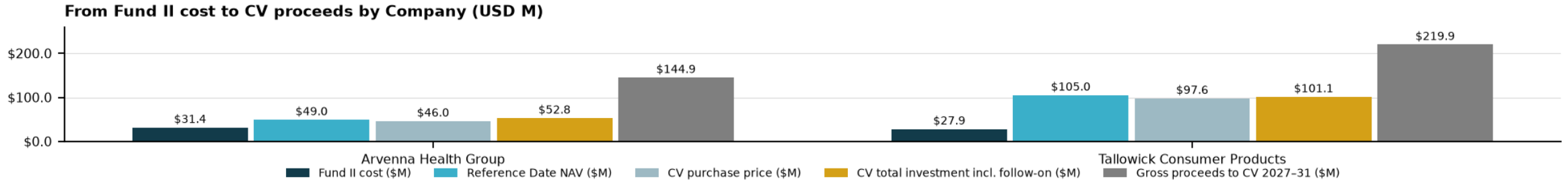
Fund II Investment Information as of 30 June 2026

Investment	Entry date	Industry	Ownership	Cost	Realised (1)	NAV	Total value	% of NAV	Gross MoIC	Gross IRR
Arvenna Health Group	Jul-2021	Healthcare	58%	$31.4M	$2.2M	$49.0M	$51.2M	31.8%	1.63x	11.2%
Tallowick Consumer Products	Nov-2019	Consumer products	74%	$27.9M	$16.3M	$105.0M	$121.2M	68.2%	4.35x	28.8%
Total				**$59.3M**	**$18.5M**	**$153.9M**	**$172.4M**	**100.0%**	**2.91x**	

Transaction Portfolio Summary

Investment	Current NAV	Purchase price (1)	Pricing to NAV	(+) Follow-on capital	Total investment	Interim proceeds	CV exit proceeds	Gross MoIC (1)
Arvenna Health Group	$49.0M	$46.0M	94%	$6.7M	$52.8M	$6.6M	$138.2M	2.90x
Tallowick Consumer Products	$105.0M	$97.6M	93%	$3.5M	$101.1M	$23.5M	$196.4M	2.22x
Total	**$153.9M**	**$143.6M**	**93.3%**	**$10.2M**	**$153.8M**	**$30.1M**	**$334.6M**	**2.47x**

Gross MoIC follows the Transaction Model convention (distributions divided by contributions on net annual cash flows) and is before buy-side fees, management fees and carried interest; including the $1.4M of buy-side fees the fund-level gross MoIC is 2.44x and the gross IRR 20.2% (exit assumed 31 December 2031). Net returns to new LPs after fees and the tiered carry are 2.00x / 16.7%.



$153.9M	**$143.6M**	**$153.8M**	**$364.7M**	**2.44x**	**20.2%**
Reference Date NAV of the Portfolio	Purchase price (93.3% of NAV)	Total CV investment incl. follow-on	Gross distributions to the CV	Gross MoIC (incl. buy-side fees)	Gross IRR

Notes: USD M; Ref. Date 30-Jun-2026. (1) Realised = fund-level dividends received (GP Q2'26 report); gross IRRs per the same report. Pricing at 94% / 93% of NAV agreed with the LPAC; MoIC per Entry & Exit tabs.

Transaction Sources & Uses and Preliminary Key Terms

$167.7M of total commitments: $145.1M funded at closing, $10.2M follow-on and $12.4M fees and expenses

Transaction Sources & Uses

Sources	Total
Total CV commitments	**$167.7M**
Funded capital raised at closing	$145.1M
of which rolling Fund II LPs	*$59.5M*
of which reinvested GP economics	*$5.2M*
of which GP out-of-pocket commitment (2)	*$2.9M*
of which new LPs (2)	*$77.5M*
Follow-on capital raised	$10.2M
Management fees and expenses (1)	$12.4M
Total Sources	**$167.7M**

Uses	Total
Gross purchase price	$143.6M
Arvenna Health Group	*$46.0M*
Tallowick Consumer Products	*$97.6M*
Follow-on capital	$10.2M
Arvenna Health Group	*$6.7M*
Tallowick Consumer Products	*$3.5M*
Management fees and expenses (1)	$12.4M
Buy-side transaction fees (1.0%)	$1.4M
Total Uses	**$167.7M**

Funded vs. unfunded

$145.1M is drawn at closing; $22.7M is unfunded at closing and drawn as follow-on capital is deployed (2027–2030) and as management fees fall due.

Transaction Preliminary Key Terms

Term	Description
Vehicle	Dunecrest Continuation Fund, a limited partnership managed by Kessport Frontier Managers Limited; single class of interests for rolling and new LPs
Fund term	Five years from closing plus two one-year extensions at the GP's discretion (with LPAC consultation)
Pricing	94% of NAV for AHG and 93% of NAV for Tallowick; NAV at 8.0x / 7.0x LTM Q2'26 EBITDA per the Transaction Model
Management fee	1.75% p.a. on net invested capital (contributed capital less capital returned), payable quarterly
Unfunded capital	$10.2M of follow-on commitments for identified add-on acquisitions plus $12.4M of fees and expenses
Preferred return	8% net IRR and 1.0x net MoIC to LPs before any carried interest
Carried interest	10% above the preferred return; 15% once LPs have received a 15% net IRR and 2.0x; 20% once LPs have received a 20% net IRR and 2.5x; 100% GP catch-up at each tier
GP alignment	2% out-of-pocket GP commitment ($2.9M) plus $5.2M of Fund II carried interest reinvested in the CV
Governance	LPAC of up to five members; conflicts approved by the Fund II LPAC; independent fairness opinion on pricing

Notes: USD M; Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026. (1) Fees and expenses: a source and a use, funded by LP contributions over the CV life. (2) GP commitment shown separately; grouped with new investors in the Transaction Model.

Transaction Process and Timeline

Six weeks of first-round diligence from VDR opening; initial indications due 23 October 2026; closing targeted for 31 December 2026

Transaction Timeline

Transaction item	Sep-26	Oct-26	Nov-26	Dec-26
Teaser distributed / VDR access (14 Sep)	★			
Investor due diligence (six weeks)	■■	■■■		
Initial indication deadline (23 Oct)		★		
Second-round diligence (select group)			■■■	
Fund II LP election window closes (27 Nov)			★	
Signing and closing (31 Dec)				★

VDR Index

Folder	Documents
1. Transaction documents	Project Dunecrest – Transaction Model vF; Project Dunecrest – Transaction Teaser vF
2. Fund documents	Fund II audited financial statements FY2025; Fund II quarterly reports Q1'26 and Q2'26 (KFF II GP Q2'26)
3. Portfolio company documents	AHG – Management Presentation (February 2026); Tallowick – Management Presentation (July 2026)
4. Market information	Macro update: Country A & Country B (Q3 2026)

Summary of Transaction Process

- A. First round
  - Prospective investors receive this Teaser, the Transaction Model and VDR access on 14 September 2026 and have approximately six weeks to complete first-round diligence.
  - Initial indications are due by 17:00 (local time, Country C) on 23 October 2026 and should state the proposed commitment, any views on pricing, and the assumed proportion of rolling Fund II LPs.
- B. Sponsor access
  - The Sponsor is available for 60-minute investor meetings with the deal teams and, on request, with the Companies' CEOs and CFOs; requests are coordinated by the Sponsor's investor relations team.
- C. Second round and closing
  - A select group is progressed to second-round diligence with access to the full data room, the fairness opinion and the draft LPA and transfer documents.
  - The Fund II LP election window runs in parallel; closing is targeted for 31 December 2026 subject to Fund II LPAC approval and customary conditions.

Contents of the initial indication

Item	Requested detail
Commitment	Proposed commitment in USD and any minimum / maximum
Pricing	Confirmation of, or proposed adjustment to, the 94% / 93% NAV pricing
Roll assumption	Assumed proportion of Fund II LPs rolling (base case 41%)
Terms	Comments on fee, carry tiers, GP commitment and governance
Diligence	Outstanding diligence items and expected timing
Approvals	Internal approval process and conditions to a binding offer

Notes: Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026; USD M unless indicated; LCA/LCB at 0.235/0.0125 USD. No assurance projections will be realised.

Project Dunecrest Contact Information

All enquiries, meeting requests and indications should be directed to the Kessport Frontier deal team below

- The Sponsor's transaction team arranges investor meetings and manages VDR access; a secondaries adviser (the "Adviser") supports the process and will circulate the second-round process letter.
- Please direct all enquiries to the individuals listed below rather than to the Companies or their management teams.

Kessport Frontier Managers Limited – Headquarters, Country C

Hendrik van Roon	Managing Partner	+00 (555) 010-0101	hendrik.vanroon@kessportfrontier.example
Amina Zerhouni	Partner	+00 (555) 010-0112	amina.zerhouni@kessportfrontier.example
Marc Tellenbach	Chief Financial Officer	+00 (555) 010-0118	marc.tellenbach@kessportfrontier.example
Youssef Oualid-Semmar	Principal	+00 (555) 010-0127	youssef.oualid-semmar@kessportfrontier.example
Sofia Brennhaug	Investor Relations	+00 (555) 010-0134	sofia.brennhaug@kessportfrontier.example

Data room

VDR access requests: sofia.brennhaug@kessportfrontier.example. Website: www.kessportfrontier.example.

Notes: Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026; USD M unless indicated; LCA/LCB at 0.235/0.0125 USD. No assurance projections will be realised.

LP Election Mechanics, Governance and Alignment

Fund II LPs choose between cash and a roll on identical terms; the GP commits $8.1M of its own capital and economics

LP Election Mechanics

- Status quo option: the Fund II LPAC will confirm that LPs may sell, roll or split their interest; no LP is required to roll and no LP is disadvantaged by rolling (identical fee, carry and governance terms to new investors).
- Cash option: selling LPs receive their pro rata share of the purchase price at closing, less their share of Fund II transaction costs (capped at 0.5% of proceeds).
- Roll option: rolling LPs contribute their Fund II interest in kind and receive CV interests at the same price per unit as new LPs; rolled capital counts towards net invested capital for the management fee.
- Base case: 41.0% of funded capital is rolled ($59.5M); a roll rate below 25% would be back-stopped by the new investors on a pro rata basis.
- Election timetable: election forms are circulated on 30 October 2026 after the second-round price is confirmed; the window closes on 27 November 2026.

Governance & Alignment

Item	Term
GP out-of-pocket commitment	$2.9M (2% of funded capital), drawn pro rata with LPs
Reinvested GP economics	$5.2M of Fund II carried interest crystallised at closing is reinvested in the CV
Total GP exposure	$8.1M (5.6% of funded capital)
Carry vesting	Carried interest paid only after LPs receive return of capital and the 8% preferred return; escrow of 20% of carry until final liquidation
Conflicts	Fund II LPAC approval of pricing; fairness opinion from an independent valuation firm; Sponsor abstains from LPAC votes on the Transaction
Reporting	Quarterly reports within 60 days, audited annual accounts within 120 days, annual LP meeting
Key person	Hendrik van Roon and Amina Zerhouni; suspension of the follow-on commitment period if both cease to devote substantially all their time

Effective carry in the base case

Modelled GP carried interest of $28.2M equals 14.6% of total profits of $192.8M (SPV Waterfall); LPs receive 2.00x net.

Notes: Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026; USD M unless indicated; LCA/LCB at 0.235/0.0125 USD. No assurance projections will be realised.

Section 2

Sponsor Overview

Sponsor Overview

A North Africa-focused mid-market investor combining operational involvement with catalytic capital; Fund III held a first close in November 2025

Overview

- Kessport Frontier Managers Limited invests growth and buy-out capital of USD 15–45M in mid-market companies across North Africa, taking significant minority or control positions with board representation and an operating partner assigned to every company.
- Since 2009 the Sponsor has invested USD 412M across Fund I, Fund II and co-investment vehicles, returned USD 231M to investors and completed seven full realisations; Fund I is fully realised at 1.9x gross, Fund II (2017 vintage, USD 185M) is in its harvesting period with the two Companies as its remaining assets, and Fund III held a USD 120M first close in November 2025.
- Three tenets underpin the strategy: strategic support (institutionalising founder-led companies), catalytic capital (crowding in development finance and local pension money) and shared ownership (employee share plans in every portfolio company).

Investment Thesis

- Large addressable markets with structural demand drivers (demographics, urbanisation, formalisation)
- Defensible business models with pricing power and local scale
- High-quality management teams willing to institutionalise
- Key sectors: healthcare, consumer goods & services, education, financial services

Key Stats

$412M Total capital invested (incl. co-invest)	**$231M** Capital returned to LPs	**9,400** Jobs supported through portfolio
7 Companies exited	**$188M** FY2025A revenue of the two Fund II Companies	**21%** FY2025A net income growth, Fund II Companies

Sponsor Team

The five senior members of the team have worked together for an average of nine years; investment committee decisions require the approval of the two independent members.



Hendrik van Roon
Managing Partner
Founded the firm in 2009; 24 years in emerging-market private equity and development finance.



Amina Zerhouni
Partner
Leads healthcare and education; board member of AHG since 2021.



Marc Tellenbach
Chief Financial Officer
Fund finance, valuation and reporting; formerly fund administrator.



Youssef Oualid-Semmar
Principal
Consumer sector lead; board observer at Tallowick.



Sofia Brennhaug
Investor Relations
LP relations, ESG reporting and fundraising for Fund III.



Independent IC members (2)
Non-Executive IC Chairman; Independent IC Member
Former regional bank CEO and former DFI portfolio director.

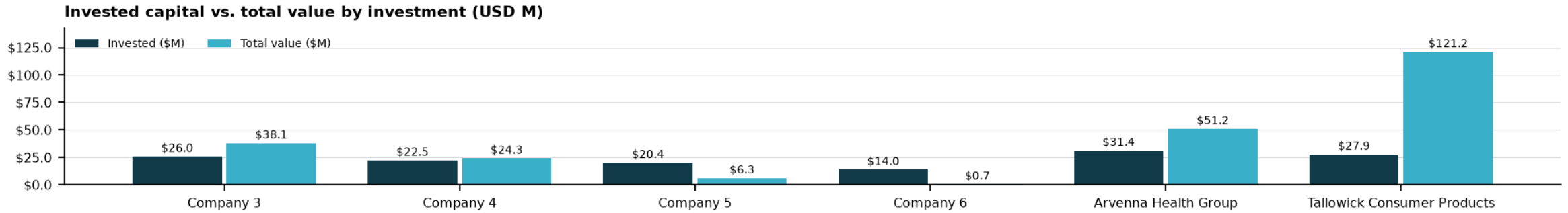
Notes: Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026; USD M unless indicated; LCA/LCB at 0.235/0.0125 USD. No assurance projections will be realised.

Fund II Track Record

Fund II: 2017 vintage, USD 185M, 92% called, DPI 0.40x, TVPI 1.38x and net IRR 6.6% at LP level; 1.70x / 11.8% gross at investment level – the two Companies are 92% of remaining NAV

Fund II Track Record as of 30 June 2026 (gross, USD)

Company	Entry	Exit	Geography	Sector	Stake	Invested	Realised (1)	NAV	Total value	Gross MoIC	Gross IRR
Company 3 (logistics; exited 2019)	Dec-2017	Nov-2019	Country A	Logistics	35.0%	$26.0M	$38.1M	–	$38.1M	1.47x	22.0%
Company 4 (agriculture; exited 2023)	Nov-2018	Sep-2023	Country B	Agriculture	42.5%	$22.5M	$24.3M	–	$24.3M	1.08x	1.6%
Company 5 (packaging; exited 2022)	Apr-2019	Dec-2022	Country A	Packaging	30.0%	$20.4M	$6.3M	–	$6.3M	0.31x	(27.6%)
Company 6 (retail; written off 2024)	Mar-2020	Apr-2024	Country B	Retail	48.0%	$14.0M	$0.7M	–	$0.7M	0.05x	(51.6%)
Subtotal realised						**$82.9M**	**$69.4M**	**–**	**$69.4M**	**0.84x**	**n.m.**
Arvenna Health Group	Jul-2021	–	Country A	Healthcare	58%	$31.4M	$2.2M	$49.0M	$51.2M	1.63x	11.2%
Tallowick Consumer Products	Nov-2019	–	Country B	Consumer products	74%	$27.9M	$16.3M	$105.0M	$121.2M	4.35x	28.8%
Subtotal unrealised						**$59.3M**	**$18.5M**	**$153.9M**	**$172.4M**	**2.91x**	
Fund II total (investment level)						**$142.2M**	**$87.9M**	**$153.9M**	**$241.8M**	**1.70x**	**11.8%**



Fund II summary (LP level, net of fees and expenses)	Value
Vintage / size	2017 / USD 185.0M
Paid-in capital	92% ($170.2M)
Distributed / DPI	$68.0M / 0.40x
NAV (incl. $13.3M net current assets) / RVPI	$167.3M / 0.98x
Net TVPI / net IRR	1.38x / 6.6%
Gross MoC / gross IRR (investment level)	1.70x / 11.8%

Notes: USD M; as of 30-Jun-2026 per the KFF II GP Q2'26 report. (1) Net exit proceeds plus fund-level income. Table gross of fees, expenses and carry; Company NAV per the Transaction Model; other NAV is escrow and cash.

Fund I Case Studies: Sector Experience in Healthcare and Consumer Goods

Two Fund I realisations – a hospital platform exited through a continuation vehicle and a consumer-goods company sold to a strategic buyer – shaped the playbooks applied to AHG and Tallowick

Case study 1 – hospital platform (Country A)

Hospital platform, Fund I (2012–2021)

The Sponsor founded a hospital holding around a single 90-bed facility and rolled up three regional clinics into an institutionalised platform. The company was exited in 2021 through a single-asset continuation vehicle backed by two secondary investors, returning 2.4x gross to Fund I. Sponsor value-add: (i) geographic roll-up of three clinics; (ii) integration under common clinical SOPs and one patient record; (iii) upgrade of imaging and theatre equipment; (iv) a corporate office with shared procurement, HR and revenue-cycle management; (v) transition from founder governance to an independent board and a professional CEO. The same team led the AHG investment in 2021.

2.4x	210k	$38M
Gross MoIC (USD)	Cases served annually at exit	Institutional co-investment mobilised

Case study 2 – consumer goods (Country B)

Personal-care manufacturer, Fund I (2013–2020)

The Sponsor partnered with the founding family of a personal-care manufacturer, taking a 40% stake, and institutionalised governance, financial reporting and the sales organisation. Revenue tripled through category diversification and export growth before a strategic sale to a regional consumer-goods group in 2020 at 2.1x gross. Sponsor value-add: (i) regional footprint expansion into two neighbouring markets; (ii) category diversification from two to five product lines; (iii) supply-chain and quality upgrades that won modern-trade listings; (iv) creation of 640 jobs, 52% held by women. Youssef Oualid-Semmar, who led the workstreams, is board observer at Tallowick.

2.1x	640	4.2M
Gross MoIC (USD)	New jobs created	Consumers reached at exit

Notes: Fund I figures gross of fees and carried interest; case studies are illustrative and not a guarantee of future results.

Section 3

Arvenna Health Group Overview

Arvenna Health Group – Company Snapshot

Headquartered in the capital of Country A, AHG is the largest and only integrated multi-facility private hospital network in the country

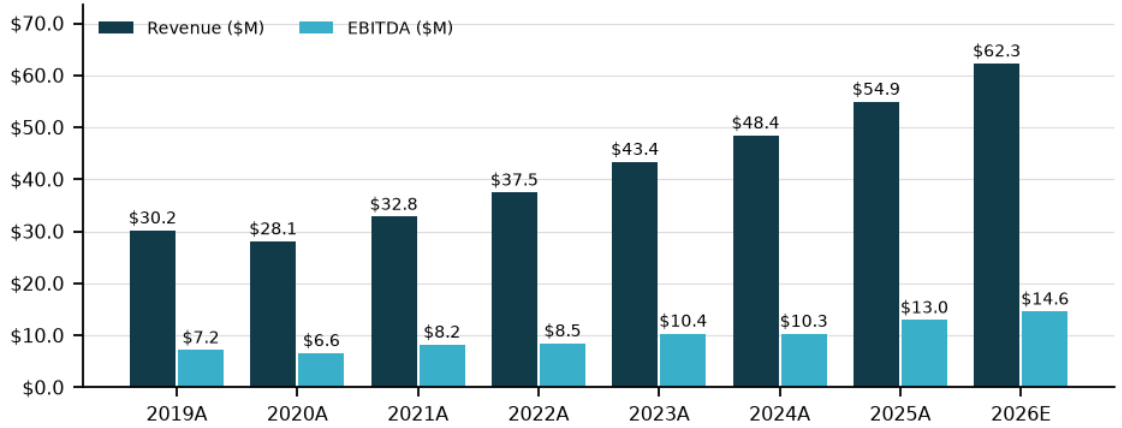
Overview

- Formed in 2019 around AHG Central (founded in 2011 by Dr. Karim Sfaxi-Mellouli), AHG operates four acute-care hospitals (AHG Central, AHG Coast, AHG North and AHG Park) and a diagnostics arm (AHG Diagnostics) with 542 inpatient beds and ~620 total beds including ICU and day-care capacity, organised around centres of excellence.
- FY2025A revenue of $54.9M and EBITDA of $13.0M (23.6% margin); revenue CAGR of 10.5% over FY2019A–FY2025A despite a weak FY2020A.
- Roughly 42% of patients and 45% of revenue come from foreign patients, making AHG the leading medical value tourism destination in the region, with private payers and out-of-pocket patients accounting for the majority of revenue.
- AHG North houses the country's only private radiotherapy unit; the group holds international hospital accreditation at three sites and laboratory accreditation at AHG Diagnostics, across ten specialties from oncology and cardiac surgery to IVF.



4 hospitals + 6 collection points; 3 sites within 15 min of the airport

Financial Overview



FY2025A Operational KPIs

4 + 1 Hospitals + diagnostics arm	**3 of 4** Owned hospital sites	**542** Inpatient beds (620 incl. ICU / day care)	**46** ICU beds
38 Operating rooms	**117k** Outpatient / ER visits	**17.7k** Surgeries	**589k** Laboratory tests (hospitals)
1,900 Staff	**310** Physicians (160 exclusive partners)	**70.3%** Bed-weighted occupancy	**46** Payer relationships

Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. Chart: organic revenue and EBITDA (6. AHG Combined Entity); FY2020A trough year.

Arvenna Health Group – Investment Highlights

Market leadership, cross-border patient flows and a funded add-on programme lift EBITDA at a 14.8% CAGR to FY2030E

Theme	Description	Key metric
Market growth	Private healthcare spending in Country A is growing at c. 9% p.a. as insurance coverage broadens and the public system refers complex cases to private providers.	$2.6B regional private hospital market (2025)
Market position & differentiation	AHG operates the largest institutional hospital group in the metropolitan area, with 542 inpatient beds versus 310 for the next largest operator; the six competitors are single-site.	25% share of private beds in the metro area
Geographic reach	Cross-border patients from six regional source markets are referred through 14 partner clinics and two airline partnerships; three hospitals are within 15 minutes of the international airport.	45% of revenue from foreign patients
Service offering & specialisation	Centres of excellence in oncology (radiotherapy at AHG North), cardiac surgery, orthopaedics, IVF and neurosurgery, supported by 14 revenue-generating service lines per hospital.	30 specialised departments
Platform expansion	Bed additions at AHG Central and AHG North in FY2023 lifted capacity by 32 beds; the add-on programme funded by $6.7M of CV follow-on adds an estimated 180 beds by FY2030.	+180 beds via M&A by FY2030E
Financial performance	EBITDA grows from $13.0M (FY2025A) to $25.9M (FY2030E incl. add-ons), a 14.8% CAGR, with net leverage turning to net cash by FY2030E.	14.8% EBITDA CAGR FY2025A–30E
M&A opportunity	Four add-on acquisitions are modelled in FY2027–FY2030 (two hospitals in secondary cities of Country A and two feeder centres in neighbouring source markets) at 5.0–6.5x EBITDA.	5 target cities identified

Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. Market size, share and bed-count figures are Sponsor estimates.

Arvenna Health Group – Company Operations

Built in five steps from a single flagship hospital to a four-hospital network plus diagnostics; every site added has reached group-level margins within three years (2)

AHG Timeline of Operations

2011
AHG Central founded by Dr. Karim Sfaxi-Mellouli (then 140 beds)

2019
Group holding formed around AHG Central by the founder

2020
Acquires 80% of AHG Coast and 90% of AHG North (1); radiotherapy unit commissioned

2021
Fund II, Fund III and DFI partners invest (Jul-2021); group governance and shared services established

2022
AHG Park acquired with Fund II follow-on (Jun-2022); AHG Diagnostics carved out as a standalone entity

2023
32 beds added at Central and North; second oncology phase at North (Jun-2023 follow-on)

Group Operations (FY2025A)

	AHG Central	AHG Coast	AHG North	AHG Park	AHG Diagnostics
Ownership / tenure	Owned – 100%	Owned – 80% (1)	Owned – 90% (1)	Leased to 2039	Owned – 100%
Inpatient beds	176	120	142	104	6 collection points
Occupancy FY2025A	75.7%	66.0%	72.2%	63.5%	n.a.
Revenue FY2025A	$17.5M	$11.4M	$15.6M	$9.7M	$1.9M
EBITDA margin FY2025A	22.3%	20.9%	21.9%	22.7%	26.2%
Partnerships	International accreditation; university teaching agreement	International accreditation	International accreditation; oncology network	Insurer preferred-provider network	Laboratory accreditation; reference-lab partnership
Centres of excellence	Cardiac surgery; neurosurgery; IVF	Orthopaedics; emergency; endoscopy	Oncology & radiotherapy; cardiac cath	Maternity & IVF; physiotherapy	Pathology; imaging

Notes: USD M at 0.235 USD/LCA. (1) Ownership at acquisition; minorities $4.4M. (2) Entity figures before intercompany eliminations ($1.0M revenue) and holding-company items; combined figures per 6. AHG (Combined Entity).

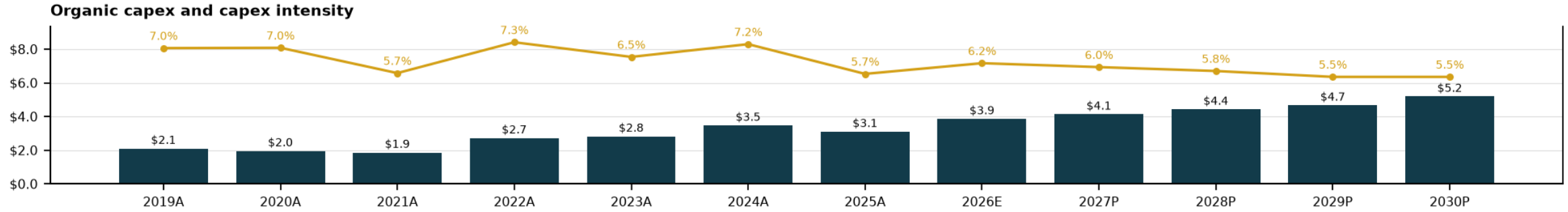
Substantial Owned Real Estate of $58.4M and a Well-Invested Asset Base

Three of the four hospitals sit on owned land and buildings independently valued at $58.4M; AHG Park is held on a 25-year lease running to 2039

	AHG Central (owned)	AHG Coast (owned)	AHG North (owned)	AHG Park (long-term leased)
Site	Capital city – central district	Coastal district, 12 min from airport	Northern suburb	Business district
Value tile	$24.6M owned real estate	$15.1M owned real estate	$18.7M owned real estate	Lease of 25 years to 2039; rent $1.1M p.a.
Built area (sqm)	21,400	13,800	17,900	11,600
Last major capex	FY2023 – 16-bed extension; 2 theatres	FY2022 – emergency department	FY2020 – radiotherapy bunker; FY2023 – oncology phase 2, 16 beds	FY2024 – imaging upgrade



Maintenance capex has averaged 6.5% of revenue over FY2021A–FY2025A ($14.0M in aggregate), keeping the theatres, imaging fleet and radiotherapy unit within manufacturer life cycles; the projection carries $3.9M in FY2026E rising to $5.2M in FY2030E.



Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. Real-estate values per the independent valuation for the Reference Date NAV.

Potential to Monetise the Real Estate Through an OpCo/PropCo Structure

A sale-and-leaseback of the three owned sites could release up to $58.4M of capital not included in the projections or the CV base case

Owned assets

Assets value by hospital	Land area (sqm)	Built area (sqm)	Construction value ($M)	Land value ($M)	Total value ($M)
AHG Central	9,800	21,400	15.9	8.7	24.6
AHG Coast	7,200	13,800	10.4	4.7	15.1
AHG North	11,500	17,900	13.2	5.5	18.7
Total	**28,500**	**53,100**	**39.5**	**18.9**	**58.4**

Market Dynamics

- Healthcare real estate in Country A trades at rental yields of 8.5–9.5%, well above the 6.0–6.5% cap rates of prime offices, attracting local insurers and pension funds constrained by regulatory allocation limits on equities.
- Investor confidence is driven by tenant quality: accredited operators with long leases and inflation-indexed rents; AHG is the only accredited multi-site tenant in the market.
- Land, equipment and certification quality plus urban locations make the three owned sites the natural seed portfolio for a listed healthcare PropCo.

AHG as anchor founder of a PropCo

- An accretive sale-and-leaseback at a 9.0% yield on a $58.4M portfolio implies rent of $5.3M p.a. against $14.6M of FY2026E EBITDA, leaving EBITDAR cover of 3.8x.
- Proceeds could repay the group's $23.6M of gross debt and fund two add-on hospitals without CV follow-on capital.
- The PropCo could later diversify into other healthcare assets (clinics, labs, senior care) and be listed on the local exchange, giving AHG a management-fee stream.

53,100 Built-up sqm	**3** Properties	**$58.4M** Total assets
100% Operational hospitals	**15 yrs** Weighted average lease expiry (target)	**9.0%** Distribution yield (target)

Not in the base case

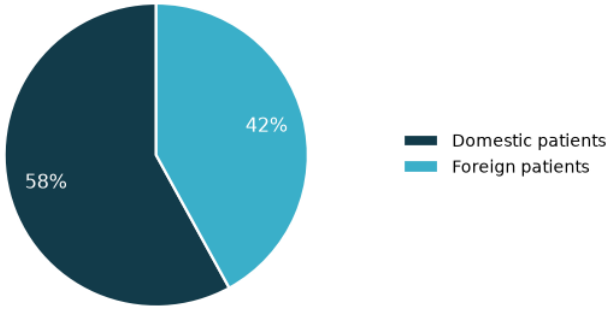
The OpCo/PropCo option is presented as upside only; neither the projections nor the CV returns assume a real-estate monetisation.

Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised.

AHG: Leader in Medical Value Tourism in the Region

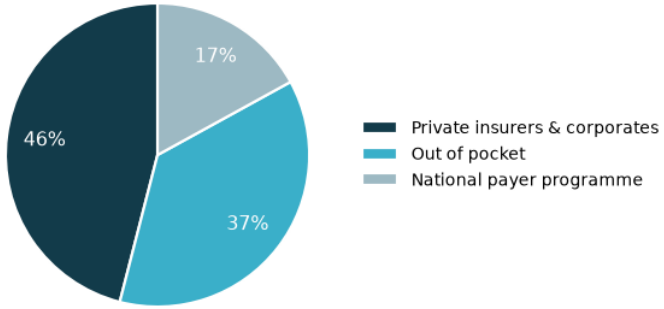
Foreign patients are 42% of admissions and 45% of revenue; six regional source markets are served through 14 partner clinics and two airline partnerships

Equal share of national and foreign patients



Foreign patients account for 42% of admissions but 45% of revenue given a higher share of surgical and oncology cases.

Revenues primarily from private payers



Potential for further geographic diversification

IMAGE REDACTED
Regional map

Category	Markets	FY2025A revenue
Domestic	Country A	$30.2M
Current MVT	6 regional source markets	$24.7M
Potential MVT	5 markets (feeder centres)	not in plan

Key comments

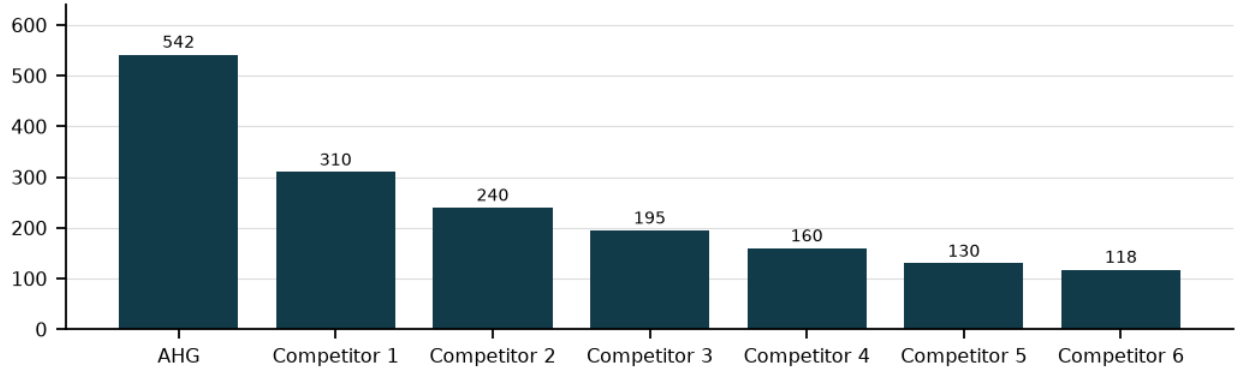
- Largest MVT group by revenue
  - Estimated 27% share of cross-border private hospital revenue in the region (1).
- Strategic geographic positioning
  - Direct flights from all six source markets; three hospitals within 15 minutes of the airport.
- Brand and payer relationships
  - 46 payer contracts including four regional insurers and two DFI-backed health schemes.
- Qualified staff
  - 310 physicians (160 exclusive partner doctors), 60% trained abroad.
- Cost advantage
  - Procedures priced at 25–35% of private tariffs in high-income markets with comparable outcomes.
- Platform for regional expansion
  - Feeder centres in two source markets modelled from FY2028 within the add-on programme.

Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. (1) Excluding one large market not served by AHG. Patient and payer splits are Company estimates.

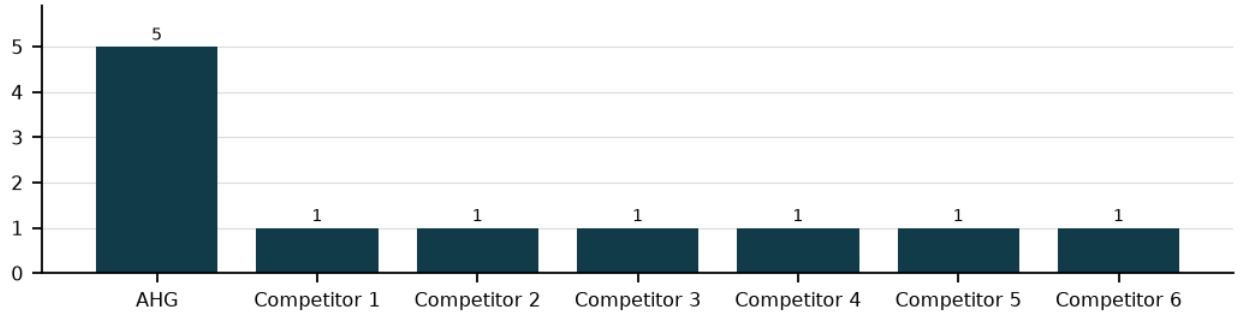
Arvenna Health Group – Competitive Landscape

AHG is the largest and only integrated private hospital group in Country A; the six competitors are single-site operators of 118–310 beds

Largest institutional hospital group in the metro area by number of beds



... and by number of facilities



Significant barriers to entry

- Brand equity and track record
  - Fifteen years of outcomes data, accreditation at three sites and a 91% patient recommendation rate.
- Economies of scale
  - Group procurement saves an estimated 6–8% on consumables and pharmacy; shared back office and one radiotherapy unit serving all sites.
- Strategic geographic coverage
  - Four hospitals across the metropolitan area plus six collection points; each competitor is confined to a single site and district.
- Partner of choice for doctors
  - Consultants gain access to a referral network, modern theatres and research partnerships; doctor churn below 5% p.a.

AHG's 542 inpatient beds represent an estimated 25% of private beds in the metropolitan area (11% nationally) and 1.7x the capacity of the next largest operator.

Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. Competitor bed and facility counts are Sponsor estimates from public registers.

Arvenna Health Group – Strategy Breakdown

Centres of excellence within a general-hospital structure: each site anchors two or three specialties and refers the rest within the network

Specialisation offering

Specialty	Central	Coast	North	Park
Oncology & radiotherapy	Visiting	–	CoE	–
Cardiac surgery & cath lab	CoE	–	Dept	–
Orthopaedics & trauma	Dept	CoE	Visiting	Visiting
IVF & maternity	CoE	–	–	CoE
Neurosurgery	CoE	–	Visiting	–
Emergency & endoscopy	Dept	CoE	Dept	Dept

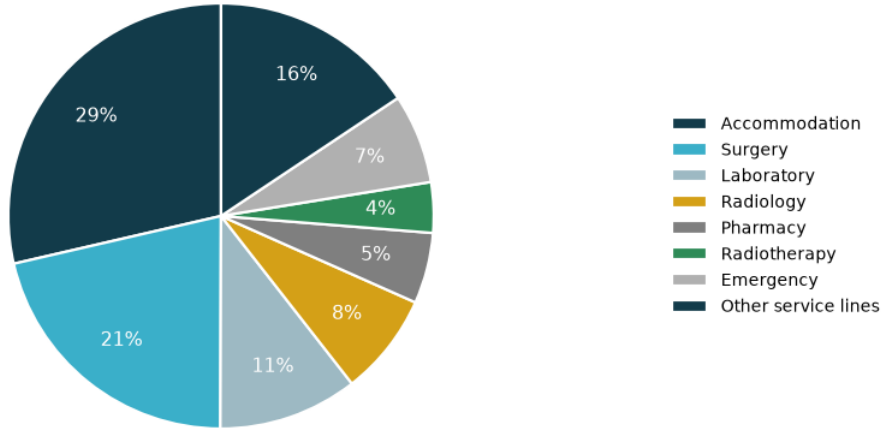
CoE = full centre of excellence; Dept = department; Visiting = visiting consultants

FY2025A breakdown by type of patient

	# patients	Revenue
Inpatients	22%	58%
Outpatients	78%	42%

Bed-weighted occupancy of 70.3% leaves headroom for the higher-acuity mix targeted in the plan.

FY2025A hospital revenue by department



Department	FY2025A ($M)	% of hospital revenue
Accommodation	15.5	28.6%
Surgery	11.5	21.3%
Laboratory	5.7	10.6%
Radiology	4.2	7.8%
Pharmacy	2.9	5.4%
Radiotherapy	2.1	3.8%
Emergency	3.7	6.8%
Other service lines	8.5	15.6%

Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. Department revenue = sum of the four hospital builds before intercompany eliminations; AHG Diagnostics excluded.

Arvenna Health Group – Best-in-Class Service Offering

A 91% patient recommendation rate, 46 direct payer contracts and consultant churn below 5% p.a. underpin the franchise

Patients are highly satisfied

- Independent patient surveys (4,200 responses in FY2025) show a 91% recommendation rate and a net promoter score of 68, both up from 84% and 55 in FY2021.
- Complaint resolution within 48 hours in 95% of cases; a group-wide patient-experience office reports monthly to the board.

91%	68
Patients who would recommend AHG	Net promoter score

Blue-chip corporates and insurers partner with AHG

AHG holds direct contracts with 46 payers: 28 corporate accounts and 18 insurers, including the four largest private insurers in Country A.

Selected payer and corporate partners

Insurer 1	**Insurer 2**	**Insurer 3**	**Insurer 4**
Corporate A	**Corporate B**	**Corporate C**	**Corporate D**

Doctors are loyal and committed

- Eminent doctors: 310 physicians practise within the group, of whom 160 are exclusive partner doctors; 60% trained abroad and 38 hold university teaching positions.
- Talent retention is a board-level priority: consultant churn is below 5% p.a. and 72% of consultants have practised with AHG for more than five years.
- Recruitment model built on four pillars: a modern working environment, centres of excellence with sub-specialty case volumes, funded attendance at international conferences, and involvement of senior consultants in hiring and marketing.

Certifications and evidenced high-quality standards

Three hospitals hold international hospital accreditation (renewed in 2024 for three years); AHG Diagnostics holds international laboratory accreditation; the group holds an international quality-management certification and the radiotherapy unit is audited annually by an international oncology network.

Accreditation and certification badges

International hospital accreditation	**Quality-management certification**	**Laboratory accreditation**
Oncology network audit	**National health ministry licence**	**Insurer quality label**

Notes: Information from AHG; survey figures per the FY2025 patient experience report.

Arvenna Health Group – Go-Forward Strategic Plan

A clear organic roadmap to consolidate AHG's leading position, before any add-on acquisition

1. Integrating the platform

- Single referral pathway and patient record across the four hospitals
- Group procurement of consumables and pharmacy (6–8% savings)
- Consultant lock-in through multi-site practising rights

2. Expanding capacity and feeder network

- Acquire large hospitals in secondary cities (add-on programme)
- Open feeder centres in two neighbouring source markets
- Border ambulatory centre for pre- and post-operative care

Organic plan – revenue

	FY2025A	FY2026E	FY2028E	FY2030E
Revenue	$54.9M	$62.3M	$76.4M	$94.4M
Growth	13.4%	13.4%	10.7%	11.2%
Bed-weighted occupancy	70.3%	71.5%	73.9%	75.7%

3. Increasing procedure complexity

- New high-complexity procedures: robotic urology, transcatheter valve procedures, complex spine
- Oncology day-care expansion at AHG North (second linear accelerator FY2027)
- Second IVF laboratory at AHG Park

4. Leveraging the position with insurers and clients

- Multi-location corporate contracts with bundled pricing
- Direct billing agreements with two further regional insurers
- Tariff indexation clauses in all renewals

Organic plan – EBITDA

	FY2025A	FY2026E	FY2028E	FY2030E
EBITDA	$13.0M	$14.6M	$18.6M	$21.9M
Margin	23.6%	23.5%	24.3%	23.2%
Capex	$3.1M	$3.9M	$4.4M	$5.2M

5. Growing CoEs and leveraging data

- Grow oncology, cardiac and orthopaedic CoEs with dedicated coordinators
- Analytics on 1.2M unique medical records for length-of-stay and outcome benchmarking (not in plan)

6. Enhancing utilisation and optimising capacity

- Lift bed-weighted occupancy from 70.3% to 75.7% by FY2030E through theatre scheduling and discharge management
- Renovation of AHG Coast wards (FY2027)
- Adoption of tele-consultation for follow-ups

$94.4M
FY2030E organic revenue

11.4%
Organic revenue CAGR FY25A–30E

Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. Plan items are Company-provided; organic case per the combined entity tab.

Arvenna Health Group – Additional Upside Through M&A

Domestic and regional inorganic growth: four add-ons modelled in FY2027–FY2030 for $21.1M of purchase price, of which $6.7M funded by CV follow-on cap

1. In Country A

- Add bed capacity in the medium term through the acquisition of two established hospitals (80–120 beds each) in secondary cities, complementing AHG's metropolitan footprint.
- Targets complement existing specialties (maternity, orthopaedics) and become referral sources for oncology and cardiac cases.
- Five target cities identified; two targets in advanced discussions at 5.0–5.5x EBITDA.

2a. Regional platform acquisition

- Acquire an existing private hospital platform in one regional source market: well-structured assets are rare, easier to integrate and immediately accretive to MVT referrals.
- One 70-bed target under review; valuation 6.0–6.5x EBITDA.

2b. Regional feeder centres

- Feeder centres re-route patients needing tertiary care to AHG while treating simpler cases locally, avoiding medical evacuations.
- Win-win for local health systems: new patients, a defined care pathway and local treatment capacity.

Modelled add-on programme

Year	Purchase price	Revenue added	EBITDA added	CV follow-on
2027	$8.4M	$7.1M	$1.4M	$2.7M
2028	$6.8M	$6.2M	$1.2M	$2.4M
2029	$3.6M	$4.5M	$0.8M	$1.0M
2030	$2.3M	$3.6M	$0.6M	$0.7M
Total	**$21.1M**	**$21.4M**	**$4.0M**	**$6.7M**

Add-ons contribute $21.4M of revenue and $4.0M of EBITDA in FY2030E (run-rate of acquisitions closed by then); the remainder of the purchase price is funded from AHG cash flow and pro rata contributions of other shareholders.

Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. Add-on figures per the combined entity acquisition block; revenue/EBITDA added are incremental to the prior year.

Arvenna Health Group – Capitalisation and Sources & Uses

The recapitalisation redeploys Fund II's capital into the CV and gives Fund II LPs liquidity; the growth plan is backed by committed follow-on capital

Pre-capitalisation	Amount	x LTM EBITDA
Adjusted net debt	**$26.0M**	**1.88x**
Gross debt	*$23.6M*	*1.71x*
(-) Cash	*($2.6M)*	*-0.19x*
(+) Adjustments (1)	*$5.0M*	*0.36x*
Equity value	**$84.4M**	**6.12x**
Kessport Frontier Fund II (58%)	$49.0M	3.55x
Kessport Frontier Fund III (12%)	$10.1M	0.73x
DFI Partner 1 (15%)	$12.7M	0.92x
DFI Partner 2 (15%)	$12.7M	0.92x
Total enterprise value	**$110.4M**	**8.00x**

Post-capitalisation	Amount	x LTM EBITDA
Adjusted net debt	**$26.0M**	**1.88x**
Gross debt	*$23.6M*	*1.71x*
(-) Cash	*($2.6M)*	*-0.19x*
(+) Adjustments (1)	*$5.0M*	*0.36x*
Equity value	**$84.4M**	**6.12x**
Dunecrest Continuation Fund (58%)	$49.0M	3.55x
Kessport Frontier Fund III (12%)	$10.1M	0.73x
DFI Partner 1 (15%)	$12.7M	0.92x
DFI Partner 2 (15%)	$12.7M	0.92x
Total enterprise value	**$110.4M**	**8.00x**

Sources

Sources	Amount	%
Continuation Vehicle – funded capital	$46.0M	87.2%
Continuation Vehicle – follow-on equity	$6.7M	12.8%
Total sources	**$52.8M**	**100.0%**

Uses

Uses	Amount	%
Purchase of Fund II's 58% interest (94% of NAV)	$46.0M	87.2%
Follow-on equity for add-on acquisitions (2027–2030)	$6.7M	12.8%
Total uses	**$52.8M**	**100.0%**

Entry valuation

LTM Q2'26 EBITDA of $13.8M (2) x 8.0x = TEV of $110.4M; equity value $84.4M after gross debt, cash, $4.4M of minorities and $0.6M of other adjustments.

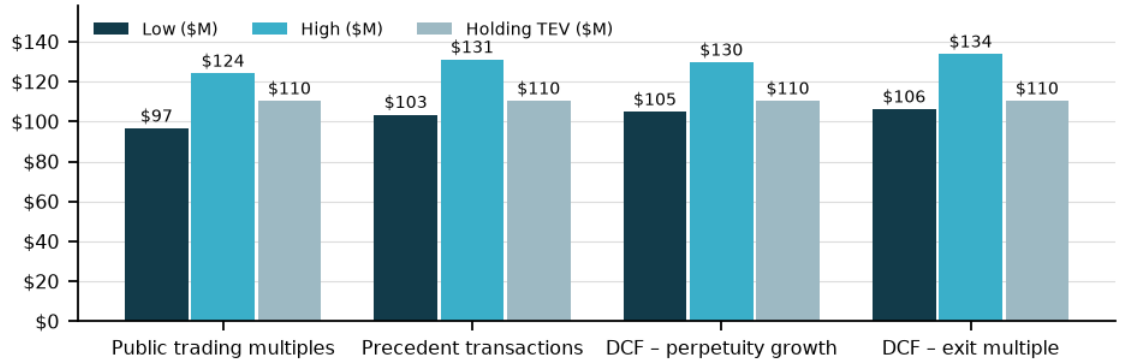
Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. (1) Minorities $4.4M and other $0.6M. (2) LTM Q2'26 EBITDA = average of FY2025A and FY2026E EBITDA.

Arvenna Health Group – Valuation and Exit Overview

Holding TEV of $110.4M (8.0x LTM EBITDA) sits in the lower half of every methodology; exit modelled at 8.5x in December 2031 via M&A

Valuation

- 1. Public trading multiples
  - 7.0–9.0x LTM EBITDA based on two listed regional hospital systems (Listed Comparable 1 and 2) trading at 7.4x and 8.8x.
- 2. Precedent transactions
  - 7.5–9.5x based on four private hospital transactions in the region since 2021, including the Sponsor's own Fund I hospital-platform exit at 8.9x.
- 3. DCF – perpetuity growth (1)
  - WACC 12–14%, terminal growth 3–4% on the organic case.
- 4. DCF – exit multiple (1)
  - WACC 12–14%, exit multiple 8.0–9.0x FY2030E EBITDA.



Exit projections (3)

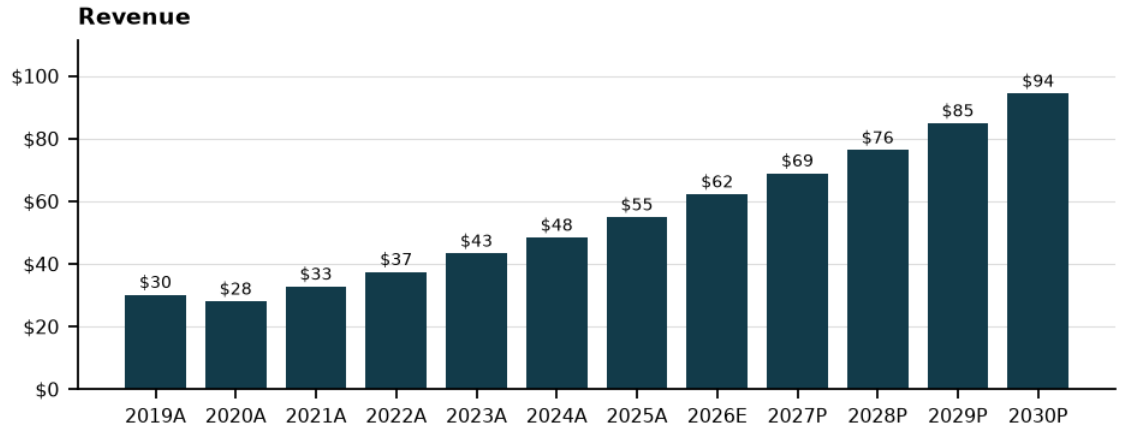
Metric	Projection
Estimated exit date	31 December 2031
Estimated exit path	M&A – strategic acquirer or regional financial sponsor
Estimated exit EBITDA multiple	8.5x FY2031E EBITDA
Exit EBITDA (2)	$29.2M
Estimated exit enterprise value	$248.1M
Total interim proceeds to CV (dividends 2027–2030)	$6.6M
CV exit proceeds (58%)	$138.2M
CV gross MoIC / IRR	2.90x / 24.2%

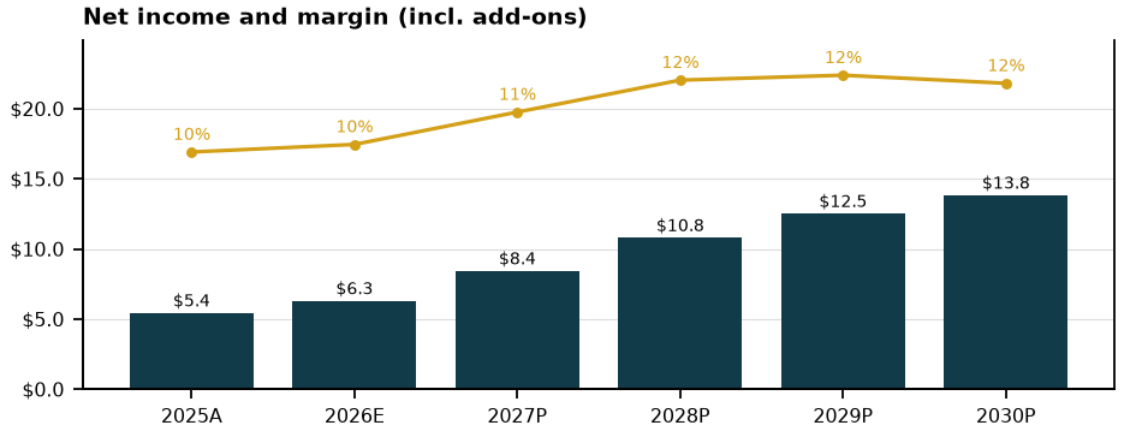
Commentary

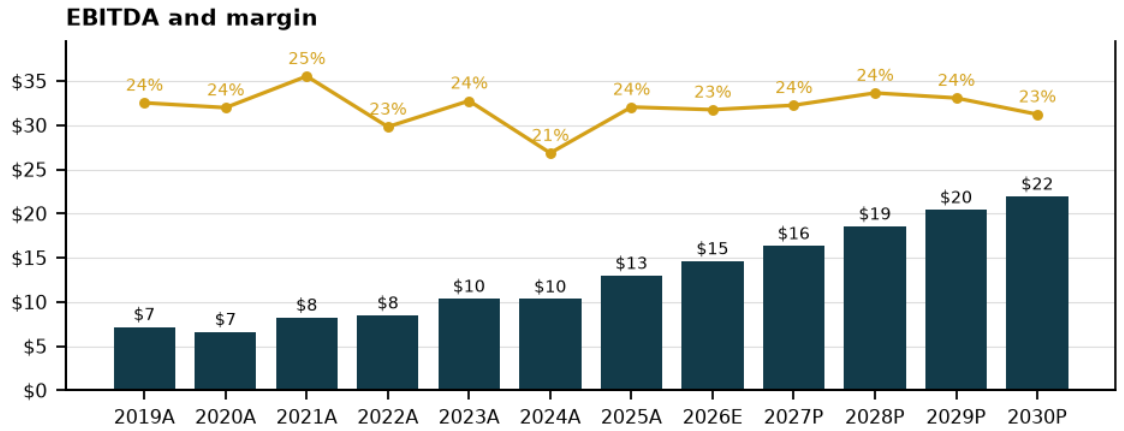
The most likely exit is a sale to a regional hospital operator or a healthcare-focused financial sponsor; a domestic listing is a secondary route given the group's scale and cash generation by FY2030E.

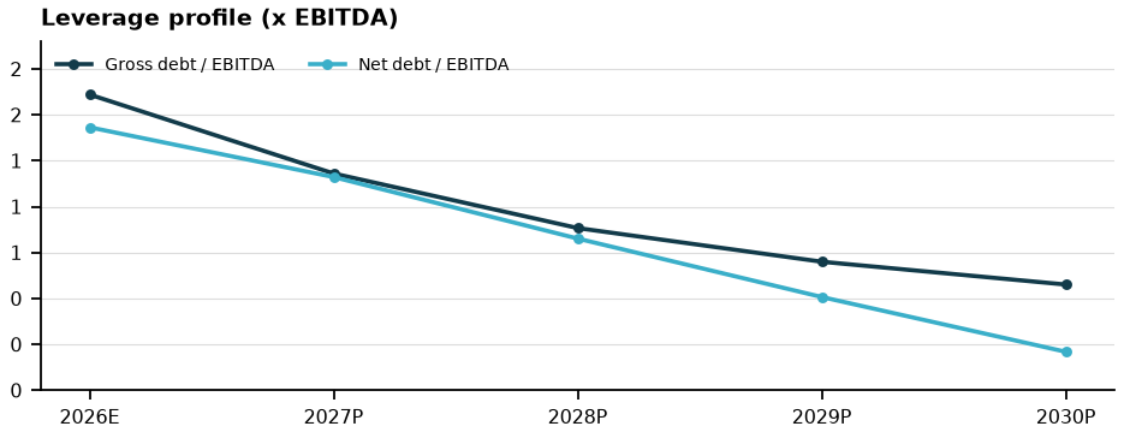
Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. Comps as of 30-Jun-2026. (1) DCF excl. add-ons. (2) FY2030E EBITDA incl. add-ons grown one year. (3) Par pricing; interim = dividends.

Arvenna Health Group – Financial Overview and Projections (Organic Growth)









Notes: USD M from LCA at 0.235 (Ref. Date); organic case unless stated; no assurance projections will be realised. Revenue/EBITDA organic; net income and leverage per the combined three-statement model (incl. add-ons); net cash from FY2030P.

Section 4

Tallowick Consumer Products Overview

Tallowick Consumer Products – Company Snapshot

Founded in 2004, Tallowick is a leading producer of branded packaged food and household products in Country B with three plants and ~2,400 staff

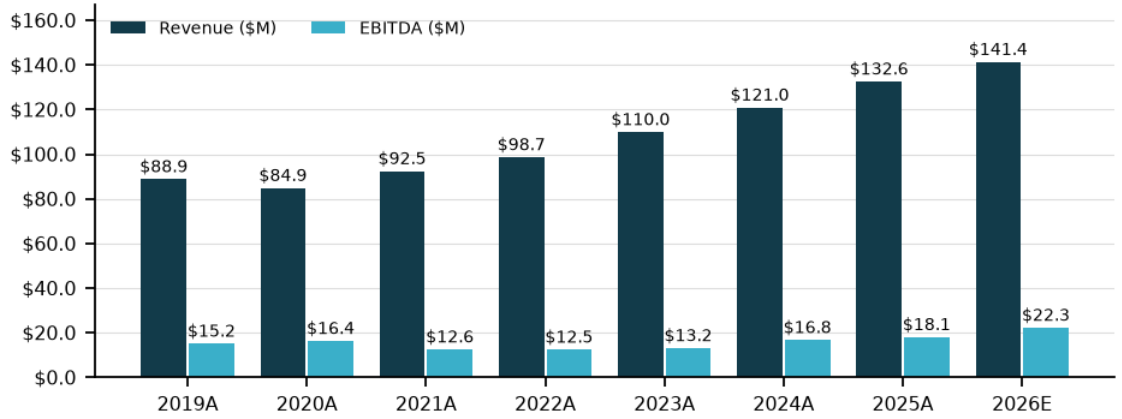
Overview

- Number-one player in dairy-style desserts and drinks (~35% volume share of the core category) and a top-three player in biscuits, wafers & snacks and in household care; Tallowick manufactures, sells and distributes its own brands through 46 exclusive distributors, 310 wholesalers and direct modern-trade coverage reaching c. 84,000 points of sale.
- Three plants (Plant 1 – capital region, 48,000 sqm; Plant 2 – coastal, 22,000 sqm; Plant 3 – inland, 15,000 sqm) with eleven production lines and c. 110,000 tonnes of annual capacity (74% blended utilisation in FY2025A); 6 hectares of expansion land at Plant 1.
- Brand portfolio: the Tallowick master brand and three category brands – Brand A (dairy-style desserts and drinks), Brand B (biscuits, wafers & snacks) and Brand C (household care). FY2025A revenue of LCB 10.6B ($132.6M) and EBITDA of $18.1M (13.7% margin); gross margin 36.2%.

Key products

Brand	Category / positioning	FY2025A volume
Brand A	Dairy-style desserts and drinks – mainstream and premium (premium sub-line launched Q4 2025)	41,200 t
Brand B	Biscuits, wafers & snacks – mainstream and premium	19,800 t
Brand C	Household care – value and mainstream	12,600 t
Tallowick master brand	Value packs across the three categories	7,900 t

Financial Overview



FY2025A Operational KPIs

$132.6M Revenue	**$48.0M** Gross profit	**$18.1M** EBITDA
$10.7M Net income	**$2.4M** Year-end net cash	**81,500 t** Tonnes sold

Notes: USD M from LCB at 0.0125 (Ref. Date); organic case unless stated; no assurance projections will be realised. Chart: revenue and EBITDA per 1. Tallowick (organic); FY2020A trough year.

Tallowick Consumer Products – Investment Highlights

Category leadership, a premium mix shift and a net-cash balance sheet support 8.5% revenue growth to FY2030E

Theme	Description	Key metric
Market positioning & diversification	Leader in dairy-style desserts and drinks with a growing presence in premium; a master brand and three category brands across three categories and three price tiers reduce dependence on any single line.	~35% share of core category
Strong financial recovery & EBITDA growth	EBITDA recovered from $12.5M in FY2022A to $18.1M in FY2025A (13.1% CAGR) as the mix shifted to premium and trade promotions were rationalised.	13.1% EBITDA CAGR FY22A–25A
Cash-driven M&A potential	Net cash of $2.4M at FY2025A rising to $30.5M by FY2030E supports the add-on programme with limited external funding.	$30.5M net cash FY2030E
Technology & production excellence	Eleven lines running on a three-shift, six-day basis at 74% blended utilisation with an 84% equipment-efficiency rate; two lines upgraded in FY2024 for premium packaging formats.	110,000 t annual capacity
Growth trajectory	Revenue forecast to grow from $132.6M to $199.1M in FY2030E including add-ons (8.5% CAGR).	8.5% revenue CAGR FY25A–30E
Market growth	The packaged-food and household-care market in Country B is growing at 7–8% p.a. driven by population growth, urbanisation and modern-trade penetration.	7.5% category market CAGR
Product mix shift & margin growth	Premium SKUs rose from 18% to 31% of volume between FY2021A and FY2025A; gross margin expanded from 36.1% to 36.2%.	31% premium share of volume

Notes: USD M from LCB at 0.0125 (Ref. Date); organic case unless stated; no assurance projections will be realised. Market size, share and capacity figures are Company estimates.

Tallowick Consumer Products – Company History

Twenty-two years from a single dessert line to a three-category, three-plant branded manufacturer; Fund II has been the majority shareholder since November 2019

2004–2010: creation	2011–2015: consolidation of the leading position	2016–2019: brand platform build-out	2019–2026: Fund II partnership and premium shift
Family-owned dessert plant in the capital region; first industrial line; minority PE investor enters (2008)	Plant 2 opened; national distribution; Brand A becomes the category leader	Brand B and Brand C launched; Plant 3 acquired; conversion to a joint-stock company	Fund II buys out the first investor (Nov-2019); premium relaunch of Brand A; export to two neighbouring markets

Corporate developments

2004	2008	2016	2019	2022	2024	2026
Founded by the Boudiaf family	Minority PE investor enters (exited 2019)	Conversion to a joint-stock company; first independent directors	Fund II buys out the first investor and subscribes new shares (74%); management retains 26%	Brand A premium relaunch; Yacine Boudiaf appointed CEO	Brand C household-care range extended; ESG policy adopted	Export listings in two markets; CV transaction proposed

Operational developments

2004	2008	2012	2015	2019	2023	2025
Line 1 – dairy-style desserts	Plant 2 – lines 2–4 (biscuits, wafers)	Line 5 – dairy-style drinks (aseptic)	Plant 3 acquired – household care lines 6–8	Line 9 – premium desserts	Lines 10–11 – premium snacks; automated packing	Solar roof at Plant 1; fleet telematics

22	3	11	1 + 3	2,400	$132.6M
Years in operation	Plants	Production lines	Master brand + category brands	Staff	FY2025A revenue

Notes: Information from Tallowick.

Tallowick Consumer Products – Company Facilities

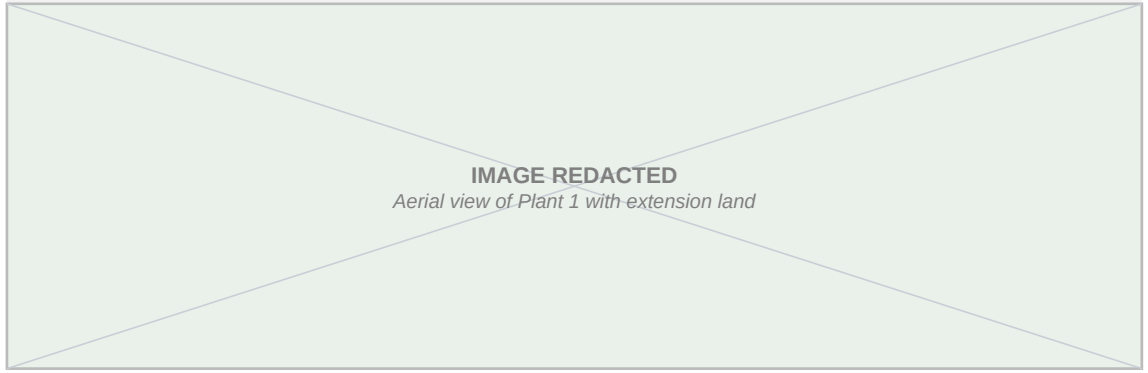
Three owned plants and eleven lines with headroom to support the plan: blended capacity utilisation of 74% in FY2025A

Facilities overview

- State-of-the-art facilities
  - Three plants certified to food-safety and quality standards; Plant 1 audited annually by two international retailers.
  - Three-shift, six-day operation on the core lines; 84% overall equipment efficiency in FY2025.
- Infrastructure overview
  - Transport and logistics: 118 delivery vehicles and 14 line-haul trucks; two regional depots.
  - Land and construction: 85,000 sqm of built surface on 21 hectares; 6 hectares of expansion land at Plant 1.
  - Utilities: 3 steam boilers, 2 water-treatment units, 1.8 MW rooftop solar (Plant 1).

Capacity and utilisation by category (FY2025A)

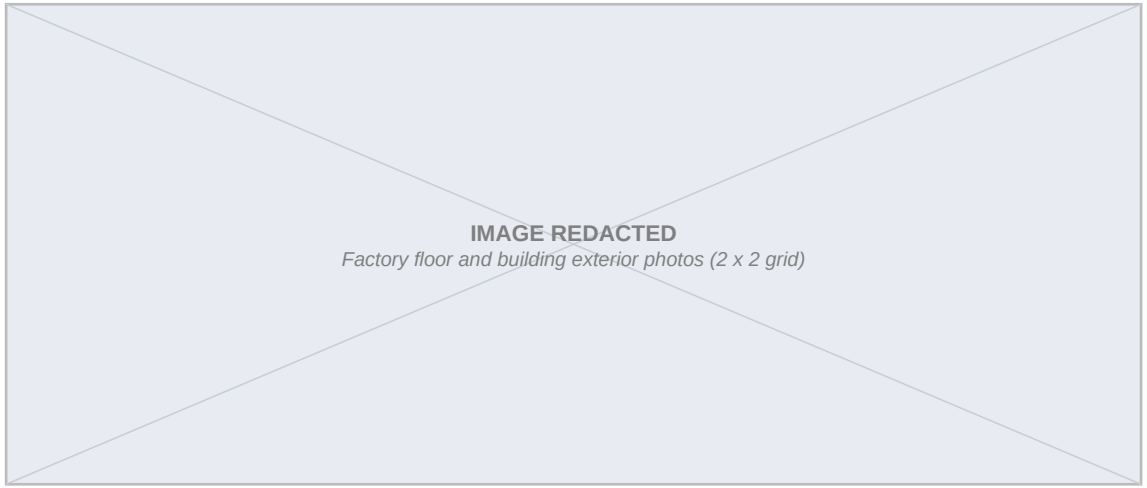
Category (lines)	Capacity (t p.a.)	Volume (t) (1)	Utilisation
Dairy-style desserts & drinks (lines 1, 5, 9)	52,000	44,100	85%
Biscuits, wafers & snacks (lines 2–4, 10–11)	38,000	24,000	63%
Household care (lines 6–8)	20,000	13,400	67%
Total	**110,000**	**81,500**	**74%**



Current facility – 48,000 sqm; Extension – 6 ha; 35 km to the capital

Historical facility acquisitions

Production line	Plant	Year	Pace (units / minute)	Supplier
Line 1 – dairy-style desserts	Plant 1	2004	650	Supplier 1
Lines 2–4 – biscuits, wafers	Plant 2	2008	3 x 720	Supplier 2
Line 5 – dairy-style drinks (aseptic)	Plant 1	2012	180	Supplier 1
Lines 6–8 – household care	Plant 3	2015	3 x 240	Supplier 3
Line 9 – premium desserts	Plant 1	2019	210	Supplier 1
Lines 10–11 – premium snacks	Plant 1	2023	2 x 900	Supplier 4



Notes: Information from Tallowick; utilisation on a three-shift, six-day basis. (1) Volumes include Tallowick master-brand value packs (7,900 t) allocated by category.

Tallowick Consumer Products – Competitive Landscape

Tallowick leads dairy-style desserts and drinks and is building a rising presence in premium, where importers have historically dominated

	Tallowick	Competitor 1	Competitor 2	Competitor 3	Competitor 4 (importer)
Market share – dairy-style desserts & drinks	35%	22%	14%	9%	11%
Market share – biscuits, wafers & snacks	19%	31%	–	12%	15%
Market share – household care	12%	–	27%	19%	22%
Product portfolio: dairy-style desserts	✓	✓	✓	✓	✓
Product portfolio: dairy-style drinks	✓	✗	✗	✗	✓
Product portfolio: biscuits & snacks	✓	✓	✗	✓	✓
Product portfolio: household care	✓	✗	✓	✓	✓
Positioning: high-end	✓ (Brand A premium sub-line)	✗	✗	✗	✓
Positioning: low-end	✓ (master-brand value packs)	✓	✓	✓	✗
Brands	Tallowick master brand; Brand A, B, C	Brand X	Brand Y	Brand Z	Imported brands
Local production	3 plants	1 plant	1 plant	2 plants	None
Imports	Premium inputs only	Finished goods (20%)	None	None	100%

Market shares are FY2025 retail-value estimates from Company panel data; competitor names withheld.

Notes: Information from Tallowick; Sponsor analysis.

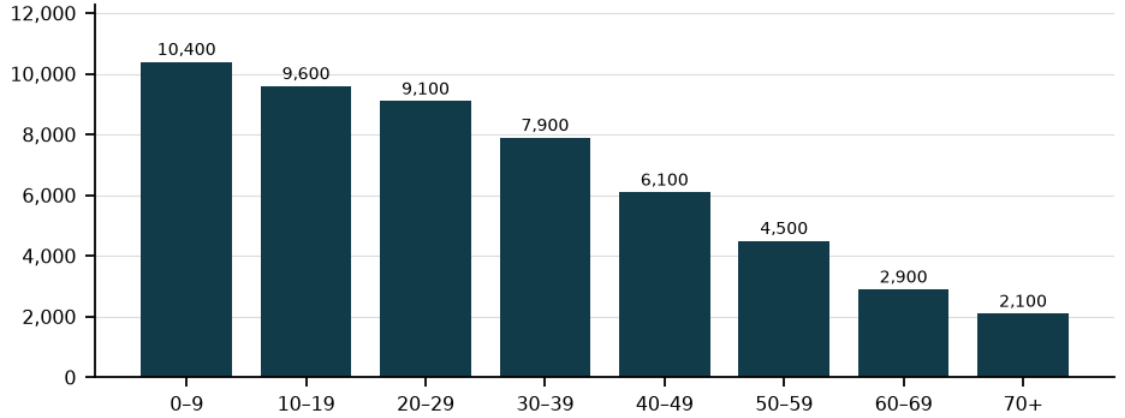
Tallowick Consumer Products – Market Overview

A large and growing packaged-food and household-care market supported by a young population, high birth rates and rising urbanisation

Overview

- Demographics
  - Population of Country B of c. 52.6M growing 1.8% p.a.; 64% urban; c. 1.4M births per year.
- Women's economic participation
  - Female labour-force participation up 6 points since 2019, lifting demand for convenience foods.
- Rising living standards
  - Living-standards indicators have improved every year since 2015; share of households in the middle-income band above 40%.
- Disposable income
  - GDP per capita growth of 3–4% p.a. in real terms (see Macro update).
- Innovation and investment
  - Four PE-backed FMCG companies in the market; premium segment growing at twice the category rate.
- Infrastructure
  - New highway corridors cut delivery times to the inland regions by a third.
- Distribution
  - Modern trade share of packaged food up from 21% to 34% in five years; c. 84,000 points of sale covered by Tallowick.

Country B population by age band (1)



Category (Country B, retail value)	2025	2030E	CAGR
Dairy-style desserts & drinks	$1.02B	$1.46B	7.4%
Biscuits, wafers & snacks	$0.86B	$1.27B	8.1%
Household care	$0.71B	$1.01B	7.3%
Total addressable	**$2.59B**	**$3.74B**	**7.6%**

Notes: Information from Tallowick. (1) National statistics office publications (accessed August 2026).

Tallowick Consumer Products – Go-Forward Strategic Plan

A growth roadmap to consolidate Tallowick's position as the leading branded platform in Country B

1. Premium product migration

- Premium willingness to pay: premium SKUs carry a 12-point gross-margin advantage
- Target 40% premium share of volume by FY2030E (31% in FY2025A)
- Brand A premium sub-line and Brand B premium snack ranges

2. Distribution expansion

- Recruit 12 further distributors in under-served inland regions
- Fleet telematics and route optimisation
- Partnerships with two pharmacy chains for Brand C

3. Marketing investment

- Marketing at 3.6% of revenue with a 60/40 pull/push split
- Focus on the high-margin premium sub-lines of Brand A and Brand B
- Trade promotions held at 5.2% of revenue

4. Operational excellence

- 24/7 operations on core lines; efficiency target 88%
- Capacity headroom of c. 25% (74% utilisation) allows growth without new lines until FY2028
- Energy cost reduction from solar and boiler upgrades

5. Competitive positioning

- Price premium of 8–10% versus Competitor 1 sustained by quality
- Mid-market launches to defend share against importers
- Modern-trade merchandising teams in 310 stores

6. Strategic capex programme

- Capex of $26.3M over FY2026E–FY2030E, partly bank-financed
- Automation of packing and warehousing
- New premium snack line in FY2028

$187.6M	16.2%
FY2030E organic revenue	Peak organic EBITDA margin (FY2028E)

Notes: Information from Tallowick; organic case per 2. Tallowick (Combined Entity).

Tallowick Consumer Products – Capitalisation and Sources & Uses

The recapitalisation redeploys Fund II's capital into the CV and gives Fund II LPs liquidity; the growth plan is backed by committed follow-on capital

Pre-capitalisation	Amount	x LTM EBITDA
Adjusted net debt / (net cash)	**($0.3M)**	**-0.02x**
Gross debt	*$3.6M*	*0.18x*
(-) Cash	*($7.6M)*	*-0.37x*
(+) Adjustments (1)	*$3.6M*	*0.18x*
Equity value	**$141.8M**	**7.02x**
Kessport Frontier Fund II (74%)	$105.0M	5.19x
Management (26%)	$36.9M	1.82x
Total enterprise value	**$141.5M**	**7.00x**

Post-capitalisation	Amount	x LTM EBITDA
Adjusted net debt / (net cash)	**($0.3M)**	**-0.02x**
Gross debt	*$3.6M*	*0.18x*
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Equity value	**$141.8M**	**7.02x**
Dunecrest Continuation Fund (74%)	$105.0M	5.19x
Management (26%)	$36.9M	1.82x
Total enterprise value	**$141.5M**	**7.00x**

Sources

Sources	Amount	%
Continuation Vehicle – funded capital	$97.6M	96.6%
Continuation Vehicle – follow-on equity	$3.5M	3.4%
Total sources	**$101.1M**	**100.0%**

Uses

Uses	Amount	%
Purchase of Fund II's 74% interest (93% of NAV)	$97.6M	96.6%
Follow-on equity for add-on acquisitions (2027–2029)	$3.5M	3.4%
Total uses	**$101.1M**	**100.0%**

Entry valuation

LTM Q2'26 EBITDA of $20.2M (2) x 7.0x = TEV of $141.5M; the Company is in a net cash position of $0.3M after $2.1M of tax on sale and $1.5M of declared but unpaid dividends.

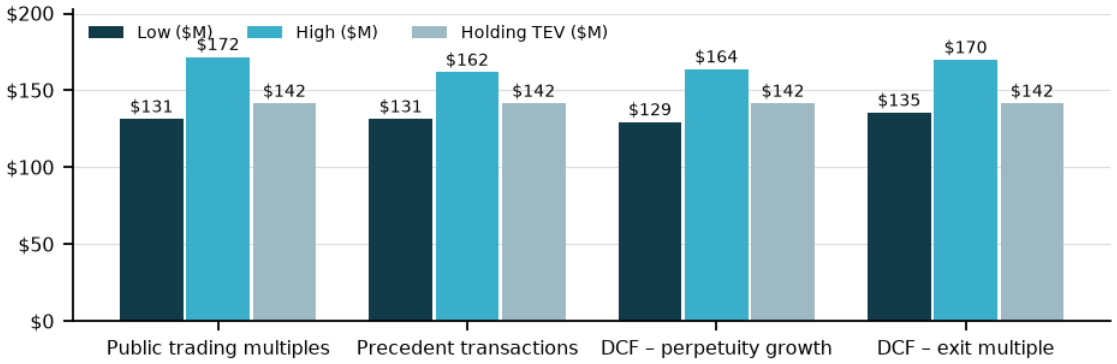
Notes: USD M from LCB at 0.0125 (Ref. Date); organic case unless stated; no assurance projections will be realised. (1) Tax on sale $2.1M and unpaid dividends $1.5M. (2) LTM Q2'26 EBITDA = average of FY2025A and FY2026E EBITDA.

Tallowick Consumer Products – Valuation and Exit Overview

Holding TEV of $141.5M (7.0x LTM EBITDA) is at the low end of every methodology; exit modelled at 7.5x in December 2031 via M&A

Valuation

- 1. Public trading multiples
  - 6.5–8.5x LTM EBITDA based on two listed regional packaged-food producers (Listed Comparable 3 and 4) trading at 6.9x and 8.3x.
- 2. Precedent transactions
  - 6.5–8.0x based on two private-equity acquisitions of regional branded-food companies in 2023 and 2025.
- 3. DCF – perpetuity growth (1)
  - WACC 13–15%, terminal growth 3–4% on the organic case.
- 4. DCF – exit multiple (1)
  - WACC 13–15%, exit multiple 7.0–8.0x FY2030E EBITDA.



Exit projections (3)

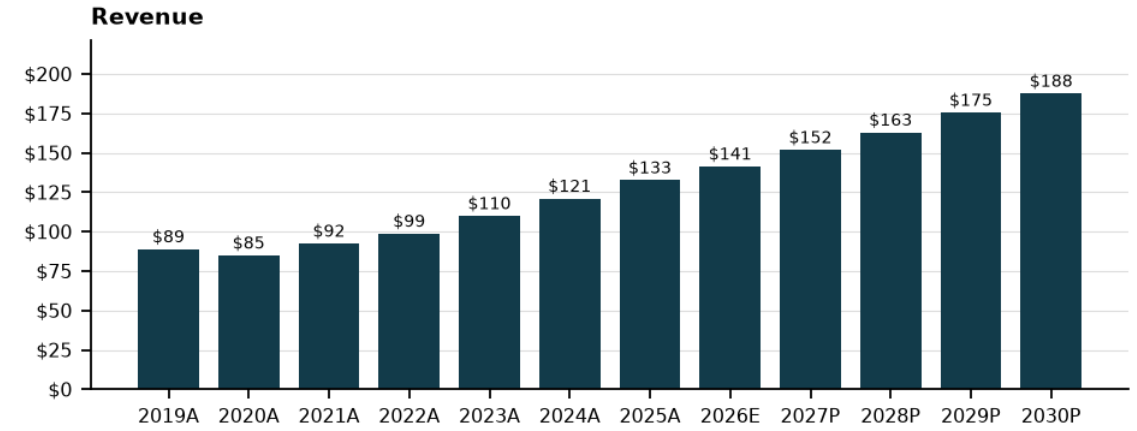
Metric	Projection
Estimated exit date	31 December 2031
Estimated exit path	M&A – regional or international consumer-goods group
Estimated exit EBITDA multiple	7.5x FY2031E EBITDA
Exit EBITDA (2)	$31.3M
Estimated exit enterprise value	$234.9M
Total interim proceeds to CV (dividends 2027–2030)	$23.5M
CV exit proceeds (74%)	$196.4M
CV gross MoIC / IRR	2.22x / 18.3%

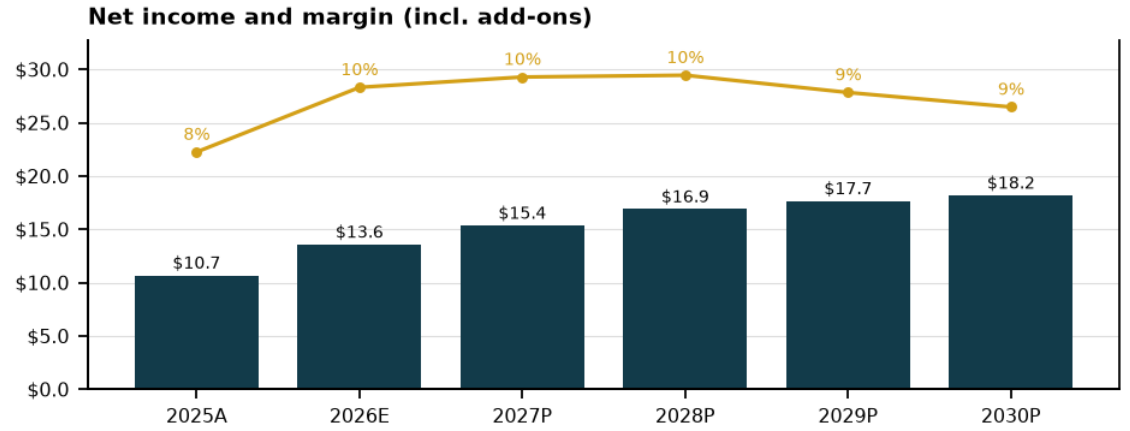
Commentary

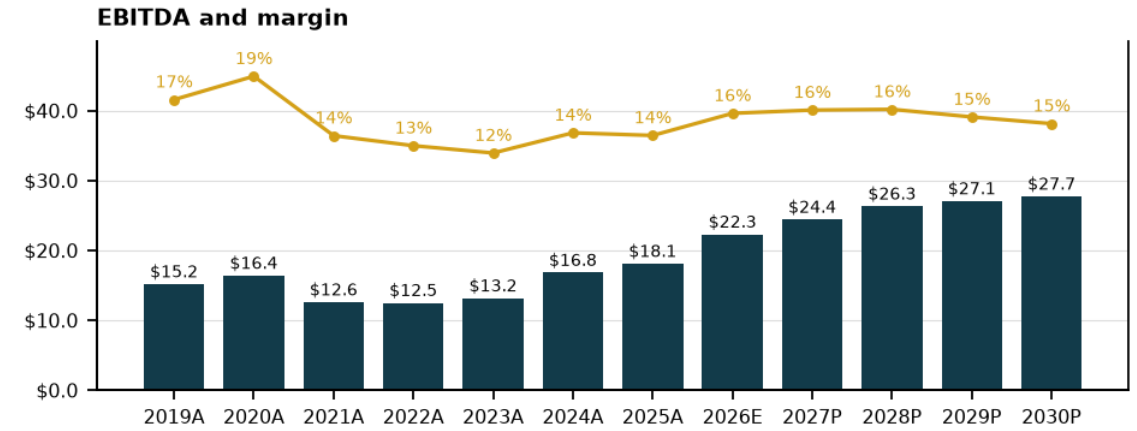
The most likely exit is a sale to a regional or international consumer-goods group seeking a manufacturing base in Country B, or to a consumer-focused financial sponsor; a domestic listing is a secondary route.

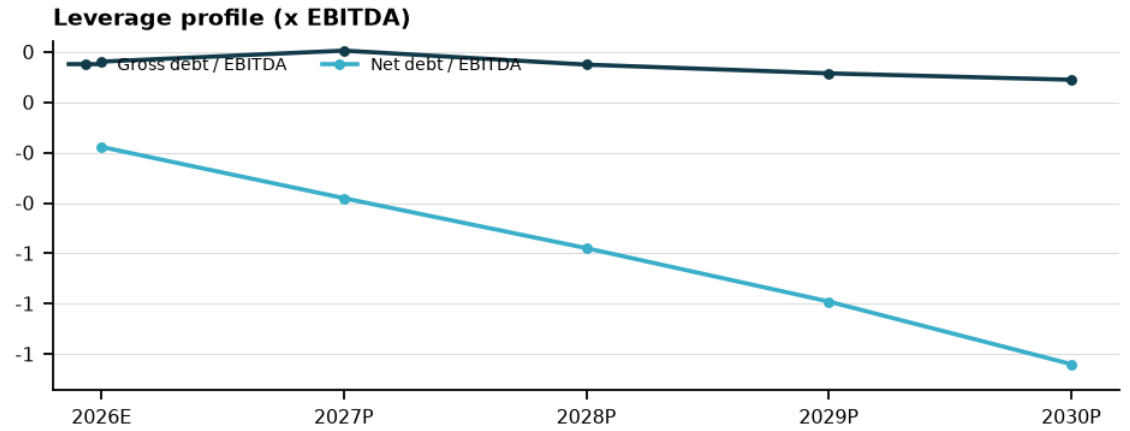
Notes: USD M from LCB at 0.0125 (Ref. Date); organic case unless stated; no assurance projections will be realised. Comps as of 30-Jun-2026. (1) DCF excl. add-ons. (2) FY2030E EBITDA incl. add-ons grown one year. (3) Par pricing; interim = dividends.

Tallowick Consumer Products – Financial Overview and Projections (Organic Growth)









Notes: USD M from LCB at 0.0125 (Ref. Date); organic case unless stated; no assurance projections will be realised. Revenue/EBITDA organic; net income and leverage per the combined three-statement model (incl. add-ons); net cash throughout.

Section 5

Continuation Vehicle Returns Analysis

Projected Exit Analysis

Both exits modelled in December 2031 via M&A at a half-turn premium to entry: AHG 2.90x / 24.2%, Tallowick 2.22x / 18.3% gross to the CV

Arvenna Health Group – Exit Analysis

Exit assumptions

Sale to a strategic acquirer or regional sponsor in December 2031 at 8.5x FY2031E EBITDA of $29.2M (0.5x above entry), giving a TEV of $248.1M; CV proceeds of $144.9M including $6.6M of interim dividends.

AHG – exit waterfall	LTM Q2'26 (entry)	FY2031P (exit)
EBITDA	$13.8M	$29.2M
EV / EBITDA	8.0x	8.5x
Enterprise value	$110.4M	$248.1M
(+) Net cash / (-) net debt	($20.9M)	($5.4M)
(+)/(-) Other (1)	($5.0M)	($4.4M)
Equity value	**$84.4M**	**$238.3M**
CV ownership	58%	58%
CV equity value	$49.0M	$138.2M
(+) Interim proceeds (2027–2030)	–	$6.6M
Total CV proceeds	**–**	**$144.9M**

2.90x
CV gross MoIC – AHG

24.2%
CV gross IRR – AHG

Tallowick Consumer Products – Exit Analysis

Exit assumptions

Sale to a regional or international consumer-goods group in December 2031 at 7.5x FY2031E EBITDA of $31.3M (0.5x above entry), giving a TEV of $234.9M; CV proceeds of $219.9M including $23.5M of interim dividends.

Tallowick – exit waterfall	LTM Q2'26 (entry)	FY2031P (exit)
EBITDA	$20.2M	$31.3M
EV / EBITDA	7.0x	7.5x
Enterprise value	$141.5M	$234.9M
(+) Net cash / (-) net debt	$3.9M	$30.5M
(+)/(-) Other (1)	($3.6M)	$0.0M
Equity value	**$141.8M**	**$265.4M**
CV ownership	74%	74%
CV equity value	$105.0M	$196.4M
(+) Interim proceeds (2027–2030)	–	$23.5M
Total CV proceeds	**–**	**$219.9M**

2.22x
CV gross MoIC – Tallowick

18.3%
CV gross IRR – Tallowick

Notes: Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026; USD M unless indicated; LCA/LCB at 0.235/0.0125 USD. No assurance projections will be realised. (1) AHG: minorities/other; Tallowick: tax, unpaid dividends. Exit EBITDA = FY2030E grown one year.

Illustrative Continuation Fund Returns for New LPs

Base case: 2.44x / 20.2% gross and 2.00x / 16.7% net to LPs after the 1.75% fee and tiered carry

Commentary

- Transaction assumptions
  - Funded capital $145.1M at closing (purchase price $143.6M plus 1.0% buy-side fees).
  - Unfunded capital $22.7M: follow-on $10.2M drawn 2027–2030 and fees & expenses $12.4M.
  - GP commitment $2.9M (2%) plus $5.2M reinvested economics.
- Continuation Fund waterfall terms
  - 1.75% management fee on net invested capital, quarterly.
  - Return of capital, then 8% preferred return (1.0x).
  - 10% carry to 15% IRR / 2.0x; 15% carry to 20% IRR / 2.5x; 20% thereafter; 100% catch-up at each tier.
- Base case assumptions
  - Net MoIC 2.00x and net IRR 16.7% to new investors (SPV Waterfall).
  - GP carried interest $28.2M = 14.6% of total profits of $192.8M.

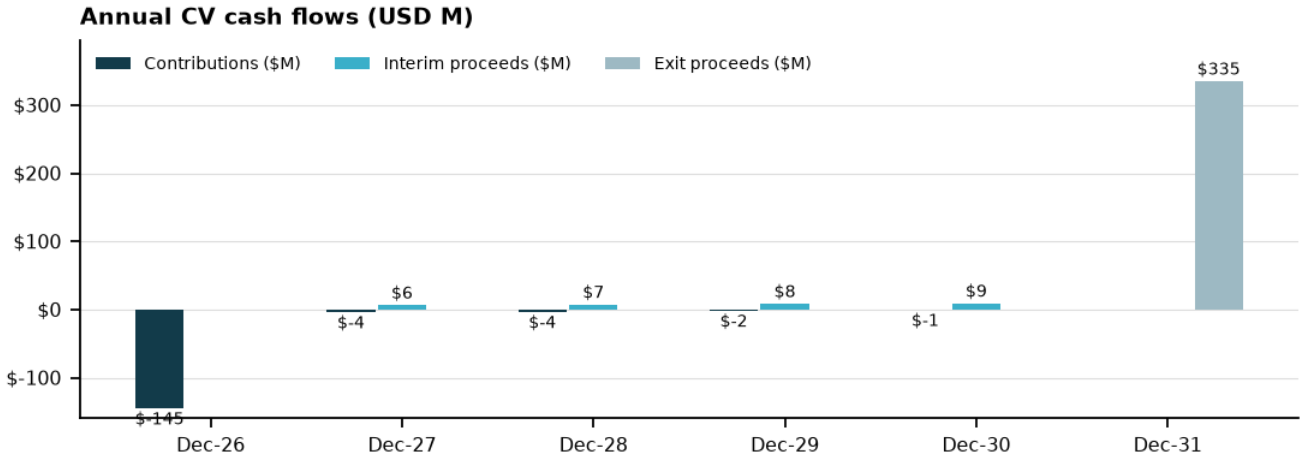
Continuation Fund cash flows (USD M)	Entry (Dec-26)	Dec-27	Dec-28	Dec-29	Dec-30	Dec-31
Initial investment – transaction fees	-1.4	–	–	–	–	–
Initial investment – AHG	-46.0	–	–	–	–	–
Initial investment – Tallowick	-97.6	–	–	–	–	–
Total initial investment	**-145.1**	**–**	**–**	**–**	**–**	**–**
Follow-on capital – AHG	–	-2.7	-2.4	-1.0	-0.7	–
Follow-on capital – Tallowick	–	-1.4	-1.3	-0.7	–	–
Total follow-on capital	**–**	**-4.1**	**-3.7**	**-1.8**	**-0.7**	**–**
Interim proceeds – AHG	–	1.1	1.5	1.9	2.2	–
Interim proceeds – Tallowick	–	5.0	5.7	6.3	6.5	–
Total interim proceeds	**–**	**6.1**	**7.1**	**8.1**	**8.7**	**–**
Exit proceeds – AHG	–	–	–	–	–	138.2
Exit proceeds – Tallowick	–	–	–	–	–	196.4
Total exit proceeds	**–**	**–**	**–**	**–**	**–**	**334.6**
Total Continuation Fund cash flows	**-145.1**	**2.1**	**3.4**	**6.3**	**8.1**	**334.6**
Gross MoIC / gross IRR	**2.44x**	**20.2%**				

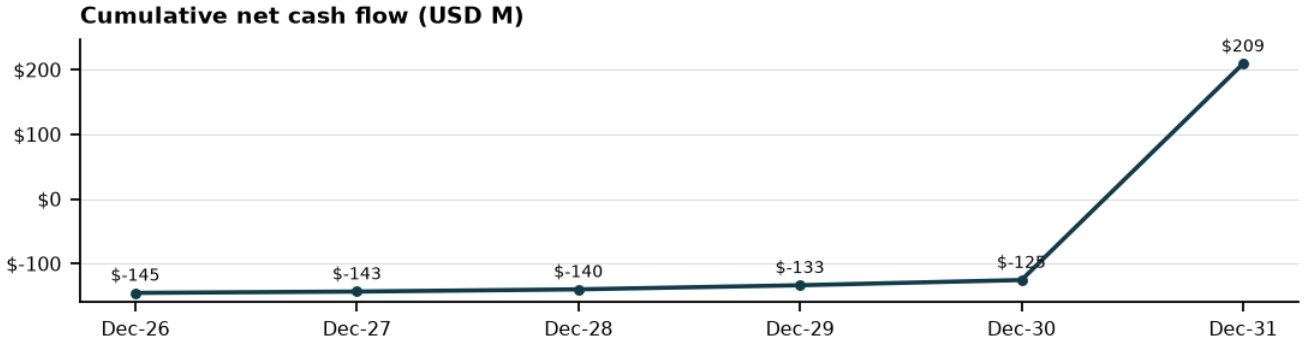
20.2% / 16.7% Gross IRR / net IRR	**2.44x / 2.00x** Gross MoIC / net MoIC	**2031** Weighted average exit year	**$152.2M** Total LP contributions (incl. fees)	**$329.2M** Total LP distributions

Notes: Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026; USD M unless indicated; LCA/LCB at 0.235/0.0125 USD. No assurance projections will be realised. Cash flows per Transaction Summary; net figures per the SPV Waterfall (LP contributions incl. fees).

Continuation Fund Cash-Flow Profile

Net cash flow is positive from 2027 as dividends exceed follow-on draws, and the 2031 exits return 2.3x the capital drawn at closing in a single year





Observations

- Peak net exposure of $145.1M is reached at closing; follow-on draws of $10.2M over 2027–2030 are more than covered by $30.1M of dividends.
- Interim proceeds grow from $6.1M in 2027 to $8.7M in 2030 as both Companies lift payout ratios on rising net income.
- Exit proceeds of $334.6M in December 2031 represent 92% of total distributions; returns are therefore sensitive to exit timing and multiple (a one-year delay at the same multiple reduces the gross IRR to 16.7%).
- Management fees on net invested capital start at c. $2.5M p.a. ($12.4M over the CV life) and are drawn from LPs quarterly; they are included in the net figures on the previous page.

$145.1M	$364.7M
Drawn at closing	Total distributions (gross)

Notes: Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026; USD M unless indicated; LCA/LCB at 0.235/0.0125 USD. No assurance projections will be realised. Contributions shown as negatives (initial investment and follow-on).

Risk Factors

Prospective investors should consider the following risks, which are not exhaustive, together with the risk factors in the CV private placement memorandum

- Concentration
  - The CV holds two assets in two frontier markets; a shortfall at either Company has a direct effect on returns.
- Currency
  - All revenue and costs are in LCA and LCB; a 10% depreciation of both currencies against the USD would reduce the modelled exit proceeds by roughly the same proportion absent local inflation pass-through.
- Exit timing and multiple
  - The base case assumes M&A exits in December 2031 at 8.5x and 7.5x; exit markets for regional assets are shallow and may require a longer hold or a lower multiple.
- Regulatory and tariff
  - Private hospital tariffs and insurance reimbursement in Country A are subject to periodic regulation; price controls on staple foods in Country B may limit pass-through of input costs.
- Execution of add-ons
  - $10.2M of follow-on capital is committed to acquisitions that are not yet signed; unspent commitments reduce returns through fees on undrawn capital only marginally, but delayed integration could weigh on EBITDA.
- Conflicts of interest
  - The Sponsor manages both Fund II (seller) and the CV (buyer); mitigated by LPAC approval, an independent fairness opinion and the LP election.
- Minority protections
  - The CV holds 58% of AHG alongside Fund III and two DFIs and 74% of Tallowick alongside management; shareholder agreements contain reserved matters that require co-investor consent.
- Key person and management
  - Both Companies depend on founder-led management teams; retention plans and succession planning are in place but not guaranteed.
- Macroeconomic and political
  - Both countries face inflation, current-account pressure and periodic political change (see the Macro update in the VDR).
- Projections
  - The projections are the Sponsor's base case derived from the Transaction Model and management budgets; actual results will differ and may differ materially.

Notes: Ref. Date 30-Jun-2026; Transaction Date 31-Dec-2026; USD M unless indicated; LCA/LCB at 0.235/0.0125 USD. No assurance projections will be realised.

Appendix – Macro Snapshot and Glossary

Summary of the Macro update: Country A & Country B (Q3 2026) available in the VDR

Macro snapshot (2025A)

Indicator	Country A	Country B
Real GDP growth	2.0%	3.1%
CPI inflation (average)	6.2%	3.9%
Policy rate (year-end)	8.00%	3.00%
Current-account balance (% GDP)	(2.1%)	(1.8%)
FX vs USD (USD per unit, Control)	0.235 (LCA)	0.0125 (LCB)
Reserves (months of imports)	4.1	12.1
Population (millions)	14.8	52.6

Implications for the Portfolio: private healthcare demand in Country A is supported by rising insurance coverage and cross-border patients paying in hard currency; Tallowick benefits from staple demand growth but faces input-cost inflation that it has historically passed through with a two-quarter lag. Both Companies are modelled in local currency and converted at the Control FX rates; no FX depreciation is assumed in the base case.

Glossary

Term	Definition
CV / Continuation Vehicle	Dunecrest Continuation Fund, the vehicle acquiring Fund II's interests
Fund II	Kessport Frontier Fund II (2017 vintage)
Fund III	Kessport Frontier Fund III; co-investor in AHG (12%)
DFI Partner 1 / 2	Development finance co-investors in AHG (15% each)
Reference Date	30 June 2026 – NAV and LTM basis
Transaction Date	31 December 2026 – closing and entry of the CV
NAV pricing	Purchase price as a percentage of Reference Date NAV (94% AHG, 93% Tallowick)
LTM EBITDA	Average of FY2025A and FY2026E EBITDA (LTM Q2'26 proxy)
Net invested capital	Contributed capital less capital returned; management fee basis
Gross / net returns	Before / after management fees, expenses and carried interest
LCA / LCB	Local currencies of Country A and Country B
A / E / P	Actual / estimate / projected fiscal year

Source: Macro update: Country A & Country B (national statistics office data and multilateral-style projections); Transaction Model Control tab.

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Kessport Frontier Managers Limited – Headquarters, Country C – www.kessportfrontier.example – +00 (555) 010-0100